

Provide Treatment for Common Throat Infections

081-833-0243

Conditions: You have a patient complaining of a sore throat. All other immediate life threats have been taken care of. You have performed a patient care handwash and taken body substance isolation, (BSI), precautions. You will need tongue blades, penlight, or other light source, pen and SF 600 (Medical Record-Chronological Record of Medical Care). You are not in a CBRN environment.

Standards: Provide treatment for common throat infections without causing harm to the patient.

Performance Steps

1. Gather equipment and supplies.
2. Identify patient and inquire about allergies.
3. Provide privacy and safety for the patient.
 - a. Place patient in position of comfort as long as procedure permits.
 - b. Explain procedure to patient.
4. Perform a head, ears, eyes, nose, and throat (HEENT) examination. (See task 081-833-0254.)
5. Identify throat disorders.
 - a. Upper respiratory infection (URI) – acute infection of the upper airway.
 - (1) Common causes – URI's are usually a result of different viruses, but can be bacterial in nature.
 - (2) Signs and symptoms.
 - (a) Sore throat.
 - (b) Cough which may be either productive or non-productive.
 - (c) Sputum may be clear or purulent.
 - (d) Low-grade fever, possible.
 - (e) Nasal congestion/discharge.
 - (f) Sinus pressure.
 - (g) Appearance of a slightly reddened pharynx with mucous streaking.

WARNING: Streptococcal (strep) pharyngitis is spread by person-to-person contact with nasal secretions or saliva, often among family or household members.

- b. Streptococcal (strep.) pharyngitis–infection of the posterior pharynx and/or tonsils.

NOTE: It is most common in the late fall, winter and early spring.

- (1) Common causes–most common is Group A streptococcus bacteria.
 - (2) Signs and symptoms.

NOTE: In some individuals, strep throat is very mild with only a few of these symptoms. In others, strep throat is severe. The onset is usually sudden.

- (a) Severe sore throat.
 - (b) Fever.

- (c) Tender/swollen neck glands.
- (d) Nausea.
- (e) Malaise.
- (f) Exudate (pus) is commonly seen on reddened tonsils and pharynx.
- (g) Headache.
- (h) Chills.

c. Peri-tonsillar abscess (PTA)—bacterial infection of the tonsils, which spreads into a cellulitis and abscess.

- (1) Common causes—usually results as a complication of “strep” pharyngitis.
- (2) Signs and symptoms.

CAUTION: An abscess is a collection of pus that forms near an area of infected skin or to the soft tissue. The abscess can cause pain, swelling, and may block the throat.

(a) The abscess usually affects one side of the mouth resulting in a tonsil deviating toward the midline.

NOTE: A peri-tonsillar abscess forms in the throat tissues next to one of the tonsils.

- (b) Ear pain on the same side as the abscess.
- (c) Severe sore throat that is isolated to one side.
- (d) Spasm in the muscles of the jaw or trismus (difficulty in opening the mouth).
- (e) Difficulty swallowing saliva.
- (f) Muffled voice, often described as a “hot potato” voice (sounds as if you have a mouthful of hot potato when you talk).
- (g) Fever may be present.
- (h) Chills.

d. Infectious mononucleosis—a viral infection that presents as a sore throat.

- (1) Common cause—viral.
- (2) Signs and symptoms.
 - (a) Lymph node (gland) enlargement of the neck.
 - (b) Shaggy, white-purple tonsillar exudate.
 - (c) Abdominal pain may indicate inflammation of the spleen.

6. Provide treatment.

a. Upper respiratory infection (URI).

- (1) Referral to medical officer (MO).
- (2) A cough suppressant, decongestant, throat lozenges and analgesic for fever and body aches will be prescribed.

NOTE: Antibiotics are not indicated for URIs.

- b. Streptococcal (strep.) pharyngitis.
 - (1) Referral to MO.

NOTE: Diagnosis is confirmed with a throat culture however the MO may treat for this condition based on symptoms alone.

(2) Antibiotics will be prescribed by the MO; penicillin has been traditionally recommended and is still very effective.

CAUTION: Even though the sore throat usually gets better on its own, individuals who have strep throat must take antibiotics to prevent more serious complications of this infection, including rheumatic fever.

c. Peri-tonsillar abscess (PTA).

WARNING: This is a surgical emergency.

(1) Referral to MO immediately.

(2) Peri-tonsillar abscesses require urgent surgical drainage and antibiotic therapy.

d. Infectious mononucleosis.

(1) Referral to MO.

CAUTION: Patients are advised to avoid contact sports (football, etc.) for at least 30 days after diagnosis due to the increased risk of splenic rupture (rupture of the spleen).

(2) The MO will may order a blood test (Monospot) along with a throat culture to rule out “strep” and confirm mononucleosis.

7. Document treatment on SF 600.

Evaluation Preparation:

Setup: For training and evaluation, use another Soldier as a simulated patient complaining of a sore throat.

Brief Soldier: Tell the Soldier the simulated patient has a common throat infection that requires treatment.

Performance Measures	GO	NO GO
1 Gathered equipment and supplies.	_____	_____
2 Identified patient and inquired about allergies.	_____	_____
3 Provided privacy and safety for the patient.	_____	_____
4 Performed an HEENT examinations. (See task 081-833-0254.)	_____	_____
5 Identified throat disorders.	_____	_____
6 Provided treatment.	_____	_____
7 Documented treatment on SF 600.	_____	_____

Evaluation Guidance: Score each Soldier according to the performance measures. Unless otherwise stated in the task summary, the Soldier must pass all performance measures to be scored GO. If the Soldier fails any steps, show what was done wrong and how to do it correctly.

References

Required
SF 600

Related
None